

Media Information

09:19

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P. 001/002

**New York-Presbyterian
Queens****Diagnostic Laboratories**

56-45 Main Street
Flushing, NY
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Director of Clinical Laboratory
Tel: 718-670-1374 Fax:

Specimen Date:		Surgical Pathology Report with Addendum		QS25-14681
In Lab Date:				
Report Date:				
Patient:				
	92ND AVE		136-25 37th Ave	
	QUEENS VILLAGE, NY		Flushing, NY	
NYP MRN:	1401911369	Telephone:		
Encounter #:	316991250	Location:	NYPQ LAB OUTREACH	
DOB-Age- Sex:	- 71 -F	Copy To:	Hui Wen Huang, FNP	

CLINICAL INFORMATION:

Left breast 3:00 4 cm fn 0.5 cm mass, r/o IDC, DCIS

DIAGNOSIS:

1. Breast, left, 3:00 4 cm fn, core biopsy:
Ductal carcinoma in situ (DCIS).
DCIS is of the papillary and cribriform types with intermediate grade nuclei.
Surrounding breast tissue shows calcifications.
The result of ER testing will follow.

Note: SMM and p63 immunostains were used in the workup of this specimen.

GROSS DESCRIPTION:

1. Received in formalin, the specimen is labeled "Left breast 3:00 x 3 passes" and consists of four cores of yellow-white tissue admixed with bloody tissue ranging from 0.2 cm to 1.5 cm in length x 0.1 cm in diameter. The specimen was collected and placed in formalin at 12:07 pm on and placed into the tissue processor at 10:00 pm on 9/3/25. ES 2C.

Electronically Signed

Esther Cheng, D.O.

The attending reviewed the diagnostic slides,
and has edited the gross and microscopic portions of the report in rendering the final diagnosis.

Addendum - Queens

Date Ordered:

Date Reported:

Addendum Diagnosis

SUN, FENGMEI

OP Marker

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SUN, FENGMEI

Surgical Pathology Report

Specimen: Left breast (DCIS)
Block: 1A

RESULTS OF ANCILLARY IMMUNOPATHOLOGY STUDIES

	PERCENTAGE OF POSITIVE CELLS	STAINING INTENSITY
ESTROGEN RECEPTOR:	>95%	STRONG

INTERPRETATION: Since $\geq 1\%$ of tumor cell nuclei express estrogen receptors, this tumor is considered to be **ESTROGEN RECEPTOR POSITIVE**.

ER, PgR, HER2 and Ki-67 immunohistochemistry was performed on tissue fixed in 10% neutral buffered formalin for at least 6 hours, up to a maximum of 72 hours using the Ventana Sp1 antibody clone for ER, the Ventana1E2 antibody clone for PgR, the Ventana 4B5 antibody clone for HER2 neu and the Ventana 30-9 antibody clone for Ki-67. Cold ischemic times meet requirements specified in latest version of the ASCO-CAP guidelines. Antibody performance was initially validated by NYP/Q and is periodically verified. Interpretation of these antibodies is based on current CAP/ASCO staining criteria.

Esther Cheng, D.O.

Billing Codes: 1; 88360_Q, 88305_Q, 88342_Q, 88341_Q (Epic Private Ambulatory)

All controls for immunohistochemistry, histochemistry, histology, and cytology show appropriate reactivity. These tests were developed and their performance characteristics determined by NYP/Q. Modified procedures such as decalcification, alcohol fixation, air dried smears and frozen sections are subject to variation from the standard formalin fixed paraffin embedded specimen. They have not been cleared or approved by the U.S. Food and Drug Administration. The FDA has determined that such clearance or approval is not necessary. These tests are used for clinical purposes. They should not be regarded as investigational or research.

SUN, FENGMEI

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END OF REPORT

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Document Information

Other: Outside Pathology

PATH

00:00

Attached To:

OUTSIDE PATHOLOGY ORDER

Orders Only on with Historical Provider,

Source Information

Jaslee Quinones | Msk Pas